

Chapter 5

Infection Prevention and Control (IPC)

Intent of the chapter

The organisation implements an effective healthcare associated infection prevention and control programme. The programme is documented and aims at reducing / eliminating infection risks to patients, visitors and providers of care. The programme is implemented across the organisation, including clinical areas and support services.

The organisation provides proper facilities and adequate resources to support the infection prevention and control programme. The organisation measures and acts to prevent or reduce the risk of healthcare associated infection in patients and staff.

The organisation has an effective antimicrobial management programme through regularly updated antimicrobial policy based on local data and monitors its implementation. Programme also includes monitoring of antimicrobials usage in the organisation.

Surveillance activities are incorporated in the infection prevention and control programme.

The programme includes disinfection / sterilisation activities and biomedical waste (BMW) management.

SUMMARY OF STANDARDS

IPC.1.	The organisation has a comprehensive and coordinated Infection Prevention and Control (IPC) programme aimed at reducing / eliminating risks to patients, visitors, providers of care and community.
IPC.2.	The organisation provides adequate and appropriate resources for infection prevention and control.
IPC.3.	The organisation implements the infection prevention and control processes in clinical areas.
IPC.4.	The organisation implements the infection prevention and control processes in support services.
IPC.5.	The organisation takes actions to prevent healthcare associated Infections (HAI) in patients.
IPC.6.	The organisation performs surveillance to capture and monitor infection prevention and control data.
IPC.7.	Infection prevention measures include sterilization and / or disinfection of instruments, equipment and devices.
IPC.8.	The organisation takes action to prevent or reduce healthcare associated infections in its staff.

*This implies that the objective element requires documentation.

Summary of Objective Elements

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Objective Element	IPC.1.	IPC.2.	IPC.3.	IPC.4.	IPC.5.	IPC.6.	IPC.7.	IPC.8.
a	CORE	CORE	CORE	Commitment	Commitment	CORE	Commitment	Commitment
b	Commitment	Commitment	CORE	Commitment	Commitment	Commitment	CORE	Commitment
c	Commitment	CORE	Commitment	CORE	Commitment	Commitment	Commitment	Achievement
d	Achievement	Commitment	CORE	CORE	Commitment	CORE	Commitment	Commitment
e	Commitment		Commitment	Commitment		Achievement	Commitment	Commitment
f	Commitment		CORE	Commitment		CORE		
g	Commitment					Commitment		
h	Commitment					Commitment		
i	Commitment					Commitment		
j	Commitment							

Standards and Objective Elements

Standard

IPC.1.

The organisation has a comprehensive and coordinated Infection Prevention and Control (IPC) programme aimed at reducing / eliminating risks to patients, visitors, providers of care and community.

Objective Elements

CORE

- a. The infection prevention and control programme is documented, which aims at preventing and reducing the risk of healthcare associated infections in the hospital. *

Interpretation: The written guidance shall be directed at prevention and control of infection in all areas of the hospital and include its monitoring. The organisation shall have infection prevention and control manual (IPC manual) that shall incorporate the structure of the programme, overall aims and objectives, all processes, activities and surveillance procedures related to the programme. This shall be based on organisational priorities, current scientific knowledge, guidelines from international/national and professional bodies and statutory requirements, wherever applicable. The hospital implements strategies, education, and evidence-based activities to reduce infection risks. The periodic assessment of risk and the setting of risk reduction goals guide the infection prevention and control programme.

Reference documents could include WHO guidelines, CDC Guidelines and Manual for Control of Hospital Associated Infections, Standard Operative Procedures by National AIDS Control Organisation (NACO), Ministry of Health and Family Welfare, Government of India.

Commitment

- b. The infection prevention and control programme identifies high-risk activities and has written guidance to prevent and manage infections for these activities.*

Interpretation: The high-risk activities are identified based on scientific literature, keeping in view the potential risk of transmission of infections to the patient, and health care provider. High risk areas and procedures across the hospital shall be identified.

Examples of these activities include performing aerosol-generating procedures; handling blood and body fluid spills, specimens, and sharps; and exposure to contaminated medical devices/equipment and bio-medical waste.

Examples of high-risk areas include operation theater, ICU, bone marrow transplant, CSSD, Cath lab, labour room, laboratory, blood centre, IVF etc.

- Commitment c. The infection prevention and control programme is reviewed and updated at least once a year.**

Interpretation: The update shall be done based on newer literature on infection prevention and outbreak prevention mechanisms, infection trends and outcomes of the audit processes.

In case the annual review does not identify any opportunities for improvement, the same shall be documented in the minutes of the infection prevention and control committee meeting.

- Achievement d. The infection prevention and control programme is reviewed based on infection prevention and control assessment tool.**

Interpretation: The organisation shall use any validated tool for performing infection prevention and control assessment tool. Examples of validated tools include WHO's Infection Prevention and Control Assessment Framework at the Facility Level and CDC's Infection Prevention and Control Assessment Tool for Acute Care Hospitals.

- Commitment e. The organisation has a multi-disciplinary infection prevention and control committee, which co-ordinates all infection prevention and control activities.***

Interpretation: This shall preferably have Hospital Administrator, Microbiologist, Physician/Infection prevention and control specialist, Surgeon, Infection prevention and control Officer, Infection prevention and control Nurse(s), Nursing Manager (Nursing Supervisor), staff from central sterile services department(CSSD), operation theatre (OT), intensive care unit (ICU) and support services(s). It could also include invitees from various departments as deemed necessary. The committee shall lay down the written guidance to guide the implementation. The composition, frequency of meetings (at least monthly), the minimum quorum required, and the minutes of the meeting shall be documented.

Risk-reduction goals and measurable objectives are established by the committee at least annually and reviewed monthly.

- Commitment f. The organisation has an infection Prevention and control team, which coordinates the implementation of all infection prevention and control activities. ***

Interpretation: The team is responsible for the day-to-day functioning of infection prevention and control programme. It shall support the surveillance process and

detect outbreaks. It shall also participate in audit activity and infection prevention and control on a day-to-day basis. Infection prevention and control team shall be staffed according to the organisation's size, the level of risk of infection, and the programme's complexity and scope. However, at a minimum, the team shall at least comprise of Infection Prevention and Control Officer and Infection Prevention and Control Nurse(s). The team shall be represented in the infection prevention and control committee.

Commitment g. The organisation has designated infection prevention and control officer as part of the infection prevention and control team. *

Interpretation: The infection prevention and control officer shall be a doctor who is knowledgeable in infection prevention and control practices. It is preferable that he/she is a clinical microbiologist/infectious disease specialist. In the absence of microbiologist, a surgeon/physician can be designated as an infection prevention and control officer. The responsibilities of the Infection prevention and control officer (IPCO) are defined in the manual.

Refer to HRM.11.d.

Commitment h. The organisation has designated infection prevention and control nurse(s) as part of the infection prevention and control team. *

Interpretation: The criteria for designating shall be by qualification (registered nurse) and additional structured training. The responsibilities of the IPCN(s) are defined in the manual. The responsibilities could include surveillance of healthcare associated infections and healthcare-associated organisms, compliance monitoring (hand hygiene, transmission-related precautions, isolation, infection-specific bundles, disinfection and sterilisation procedures, and checklists), education, working on outbreaks and documentation. At a minimum, there shall be one dedicated nurse per 250 beds. They shall be supported by 'link nurses' (trained in infection prevention and control) who actively participate in infection prevention and control activities in their work areas.

Refer to HRM.12.d.

Commitment i. The organisation implements information, education and communication programme for infection prevention and control activities for the community.

Interpretation: The organisation could work with stakeholders and create information, education and communication messages. Examples of infection prevention and control activities include hand hygiene, appropriate use of antimicrobials, use of personal protective equipment etc.

Commitment j. The organisation participates in managing community outbreaks.

Interpretation: The organisation coordinates with external agencies, including statutory, to respond effectively to community outbreaks. This includes communication (both internal and external), roles and responsibilities for staff and training of staff.

Standard

IPC.2.

The organisation provides adequate and appropriate resources for infection prevention and control.

Objective Elements

CORE

a. The management makes available resources required for the infection prevention and control programme including allocation of adequate funds from its annual budget.

Interpretation: The management shall ensure that the resources required are made available on a continual and sustained manner. This includes both men and materials.

There shall be an allocation in the budget demarcated for infection prevention and control activities. This shall be prepared, taking into consideration the scope of the activity and previous years' experience.

Refer to ROM.5.c.

Commitment b. Adequate and appropriate personal protective equipment, soaps, and disinfectants are available and used correctly.

Interpretation: They shall be available at the point of use, and the organisation shall ensure that it maintains an adequate inventory.

Personal protective equipment includes:

- Gloves
- Protective eyewear (goggles)
- Mask
- Gown
- Boots/shoe covers and
- Cap/hair cover

The staff use PPE appropriate to the risks involved and guidance available. The PPE is removed as soon as the purpose is served.

CORE

c. Adequate and appropriate facilities for hand hygiene in all patient-care areas are accessible to healthcare providers.

Interpretation: The organisation provides at least one easily accessible washbasin with running water in every patient care area for health care providers. For hand wash, the organisation could consider providing large washbasins, hands-free tap, soap and facility for drying hands without contamination. Hand rub shall be available in every patient care area. Hand rub dispensers can be installed at convenient points and can also be carried by staff as they move between patients and within facilities.

Commitment d. Isolation / barrier nursing facilities are available.

Interpretation: The organisation shall define the conditions where isolation is required, and the conditions wherein barrier nursing or both are required. The same shall be carried out. The organisation shall ensure that it provides the necessary resources to carry out the activity (for example clothing, masks, gloves, etc.). Ideally, patients requiring isolation (contact, droplet and airborne) shall be placed in isolation rooms and airborne cases be kept in negative pressure rooms. The door(s) to the aisle or other rooms shall be kept closed at all times. Appropriate signage shall be used/displayed. Organisation implements isolation of patients with MRSA, VRE.

Refer to the glossary for “isolation/barrier nursing”.

Standard

IPC.3.

The organisation implements the infection prevention and control processes in clinical areas.

Objective Elements

CORE

a. The organisation adheres to standard precautions at all times. *

Interpretation: Adherence to standard precautions is one of the fundamental tenets of infection prevention and control. In every area of the organisation, standard precautions shall be adhered.

Refer to the glossary for “standard precautions”.

The components of standard precautions include hand hygiene, appropriate use of personal protective equipment, respiratory etiquettes, safe injection practices, sterile instruments and devices, clean and dis-infected environmental surfaces cleaning and disinfection of equipment, and needle-stick and sharps injury prevention. Appropriate preparation of body parts before a procedure and the use of disinfected/sterilised instruments is ensured.

CORE

b. The organisation adheres to hand-hygiene guidelines. *

Interpretation: The organisation shall adhere to international/national guidelines on hand hygiene. A good reference is the WHO guidelines on hand hygiene in health care of 2009.

The organisation could display the necessary instructions near every hand-washing area.

Commitment

c. The organisation adheres to transmission-based precautions. *

Interpretation: This shall cover airborne, droplet and contact modes of transmission. Personal protective equipment (PPE) to be used in various situations of patient care are identified and used appropriately. This shall be applicable across the organisation. Refer to international guidelines like that of CDC.

CORE

d. The organisation adheres to safe injection and infusion practices. *

Interpretation: This shall include “One needle, One syringe, Only one time” as recommended by CDC. A good reference guide is “WHO best practices for injections and related procedures toolkit”.

Commitment e. Appropriate antimicrobial usage policy is established and documented *

Interpretation: The organisation shall identify clinical conditions in which antimicrobial agents [antimicrobial (including anti-tubercular agents), anti-fungal agents, anti-viral agents and anti-parasitic agents] shall be used in terms of the type of the antimicrobial agent, monotherapy vs combination therapy, escalation and de-escalation of therapy, dose and duration of antimicrobial therapy. The organisation shall develop a system of monitoring site specific antibiograms/antimicrobial susceptibility (based on culture sensitivity) and accordingly develop its antimicrobial usage policy, which shall be reviewed at periodic intervals (at least every year) for its continuing applicability. A good reference guide to develop antimicrobial policy is step-by-step approach for development and implementation of hospital antimicrobial policy and standard treatment guidelines by WHO-2011. Ministry of Health’s National Treatment Guidelines for Antimicrobial Use in Infectious Diseases, Indian Council of Medical Research’s Treatment Guidelines for Antimicrobial Use in Common Syndromes shall be considered while framing the antimicrobial usage policy.

The organisation can also refer to national and international guidelines from professional societies while framing the policy.

It is preferable that the organisation has a standardised methodology for antimicrobial susceptibility testing.

The antimicrobial usage policy shall identify a list of restricted antimicrobial agent it also considers introducing audit and formulary restrictions.

The list of antibiotics among the restricted antimicrobials shall adhere to WHO’s AWaRe classification.

CORE

f. The organisation implements the antimicrobial stewardship programme and monitors the use of antimicrobial agents.

Interpretation: The antimicrobial agents shall be prescribed as per the organisation’s policy. The organisation needs to implement a mechanism for ordering restricted antimicrobial agents. Deviations are brought to the notice of concerned clinicians, and corrective and preventive actions are taken and documented.

The organisation shall have a mechanism to monitor the appropriate use of restricted antimicrobial agents.

Antimicrobial stewardship programme shall be monitored by antimicrobial stewardship forum consisting of clinicians.

The antimicrobial stewardship programme must aim to guide efforts to improve appropriate and necessary antimicrobial use. It shall include leadership commitment, accountability, drug expertise, action, tracking, reporting and education.

The organisation generate awareness amongst health care providers and community for AMR 2017-2021, and rational use of antimicrobials.

Standard

IPC.4.

The organisation implements the infection prevention and control processes in support services.

Objective Elements

Commitment a. The organisation has appropriate engineering controls to prevent infections.*

Interpretation: This shall include the design of patient care areas (optimum spacing between beds is one-two metres), operating rooms including zoning of OT, air quality and water supply. Refer to NABH guidelines on OT air-conditioning.

Issues such as air-conditioning plant and equipment maintenance, cleaning of air-conditioning ducts/filters, air handling units, cleaning of cooling towers, cleaning/replacement of filters, prevention of fungal colonisation shall be included. Water-supply sources and system of supply, testing for water quality must be included.

Refer to FMS.2.f., FMS.3.c. and COP.14.g.

Commitment b. The organisation designs and implements a plan to reduce the risk of infection during construction and renovation.*

Interpretation: A validated tool (for example infection prevention and control risk assessment tool)(ICRA) shall be used to identify the risk of infection during construction and renovation. Facility construction/renovation could ensure that when new facilities are built, infection prevention and control is considered from the design stage onwards. Any renovation work in the hospital shall be planned with the infection prevention and control team concerning architectural segregation, traffic flow, use of materials, etc.

Refer to FMS.1.e.

CORE

c. The organisation adheres to housekeeping procedures. *

Interpretation: Housekeeping shall be addressed at all levels of the organisation, for example ward, OT, public areas including toilets, corridors. Regular cleaning to remove visible dirt and dust is mandatory. This includes the environment, fixtures, fomites, furniture, furnishings, equipment, etc., as applicable. A risk stratification matrix may be used to determine the frequency of cleaning. The physical environment may be divided into several areas depending on the risk of transmitting microorganisms. The criteria used to identify these areas can include the number of footfalls, the type of activity performed (for example, clinical versus non-clinical) and the probability of being exposed to body fluid (for example, in an operation theatre or laboratory). It is preferable that the organisation follows a uniform policy across different departments within the organisation.

The common disinfectants used are identified as per national / international guidelines, dilution protocols are established, and its usage in the appropriate situation is complied with. It shall also include procedures for terminal cleaning, blood and body fluid cleanup and isolation rooms. A dusting of any sort inside the clinical areas shall be avoided.

A good reference is "CDC Guideline for Disinfection and Sterilization in Healthcare Facilities, 2008" and "Swachhta Guidelines for Public Health Facilities".

CORE

d. Biomedical waste (BMW) is handled appropriately and safely.

Interpretation: Proper segregation and collection of biomedical waste from patient-care areas of the hospital are implemented. Wastes to be segregated and collected in different colour-coded bags and containers as per statutory provisions. Monitoring shall be done by members of the infection prevention and control committee/team. Biomedical waste shall be handled in the proper manner using appropriate personal protective equipment.

The organisation ensures that biomedical waste is stored in accordance with statutory provisions. Biomedical waste is handed over to the authorised vendor for transport to the site of treatment and disposal. Monitoring of biomedical waste management program shall be done as per statutory requirements.

Commitment e. The organisation adheres to laundry and linen management processes. *

Interpretation: The laundry can be in-house or outsourced. The organisation shall implement its documentation regarding the change of linen. The organisation shall implement its defined process of handling linen in patient care

units, during transport to the laundry (either in-house or outsourced) and inside the laundry. Process for storage and distribution of clean linen within the organisation shall be complied. The separation of clean linen from dirty linen shall be maintained at all times. If outsourced, the organisation shall ensure that it establishes adequate controls to ensure infection prevention and control.

Commitment f. The organisation adheres to kitchen sanitation and food-handling issues. *

Interpretation: This shall be applicable even if this activity is outsourced. The organisation shall adhere to all statutory requirements, including screening of kitchen workers and food handlers. It is preferable that they also adhere to national and international guidelines while addressing this issue. Kitchen sanitation measures are implemented to prevent the risk of cross-contamination.

Food is prepared, handled, stored and distributed safely. The dietary services shall be designed in a manner that there is no criss-cross of traffic. All the activities shall fall in a sequence. The organisation shall ensure that hygienic conditions are followed throughout.

Other indicative points are:

- dedicated food storage/refrigeration areas exist to ensure food preservation;
- food storage areas/refrigerators are maintained appropriately;
- all food products are stored off the floor;
- cleaning supplies stored in a separate location away from food;
- separate dedicated food preparation areas exist;
- measures are in place to ensure that flies and insects do not come in contact with prepared/stored food;
- food distribution to patients occurs, where possible, in temperature appropriate food service trolleys (hot food kept hot and cold food kept cold).

Standard

IPC.5.

The organisation takes actions to prevent healthcare associated infections (HAI) in patients.

Objective Elements

- Commitment a. The organisation takes action to prevent catheter-associated urinary tract Infections.**

Interpretation: The basic principles of infection prevention, namely standard precautions and transmission-based precautions, shall be adhered. The organisation shall develop, implement and monitor a care bundle to prevent catheter-associated urinary tract infections. A good reference is the CDC/WHO/SHEA guidelines.

- Commitment b. The organisation takes action to prevent ventilator- associated pneumonia.**

Interpretation: The basic principles of infection prevention, namely standard precautions and transmission-based precautions, shall be adhered. The organisation shall develop, implement and monitor a care bundle to prevent ventilator-associated pneumonia. A good reference is the CDC/WHO/SHEA guidelines.

- Commitment c. The organisation takes action to prevent central line associated blood stream infections.**

Interpretation: The basic principles of infection prevention, namely standard precautions and transmission-based precautions, shall be adhered. The organisation shall develop, implement and monitor the compliance to care bundle to prevent catheter linked blood stream infections. A good reference is the CDC/WHO/SHEA guidelines.

- Commitment d. The organisation takes action to prevent surgical site infections.**

Interpretation: The basic principles of infection prevention, namely standard precautions and transmission-based precautions, shall be adhered. This shall include pre-op, intra-op and post-op measures and develop ,implement and monitor the compliance to care bundle to prevent surgical site infections. A good reference is the CDC/WHO/SHEA guidelines.

Standard

IPC.6.

The organisation performs surveillance to capture and monitor infection prevention and control data.

Objective Elements

CORE

- a. The scope of surveillance incorporates tracking and analysing of infection risks, rates and trends**

Interpretation: The organisation shall use a judicious mix of active and passive surveillance. The organisation could lay down the parameters that need to be captured and the process for reporting. The collection of surveillance data is an on-going process and is done at regular intervals (may be monthly and consolidated into an annual report), and the organisation shall take suitable steps based on the analysis. A simple calculation of infected patients (numerator) provides only limited information which would be difficult to interpret. Risk factor analysis would require information for both infected and non-infected patients to calculate infection and risk-adjusted rates.

- Commitment b. Verification of data is done regularly by the infection prevention and control team.**

Interpretation: The data collected shall be authenticated by the infection prevention and control team by going through every data or by using random sampling.

In case the collection of data is done only by the infection prevention and control team, verification is not necessary

- Commitment c. Surveillance is directed towards the identified high-risk activities.**

Interpretation: The organisation must be able to provide evidence of conducting periodic surveillance in its identified high-risk activities and high-risk areas. It shall define the frequency and mode of surveillance. The surveillance system shall be appropriate and adhering to national/international guidelines. Surveillance activities include areas where demolition, construction or repairs are undertaken.

CORE

- d. Surveillance includes monitoring compliance with hand-hygiene guidelines.**

Interpretation: The monitoring shall be done at a minimum once every month. An appropriate sample size shall be chosen and all categories of staff (involved in direct patient care) shall be monitored. The compliance levels shall be shared with the relevant staff. A good tool is the WHO's "Observation Form".



CORE



Commitment



Achievement



Excellence

Achievement e. Surveillance includes mechanisms to capture the occurrence of multi-drug-resistant organisms.

Interpretation: The organisation shall monitor the occurrence and spread of multi-drug resistant organisms (MDROs) for example methicillin-resistant *Staphylococcus aureus* (MRSA), multi-drug resistant gram-negative bacteria, Vancomycin-resistant enterococci (VRE) etc.

CORE f. Surveillance includes monitoring the effectiveness of housekeeping services.

Interpretation: Monitoring of the effectiveness of housekeeping services shall be done regularly. The organisation shall define the periodicity. This is applicable even if the housekeeping services are outsourced. It is mandatory to capture the effectiveness of the housekeeping activities and not just verify if the housekeeping activity has been done as per the defined frequency. To capture effectiveness, the organisation shall identify desired outcome parameters for housekeeping activities.

For example, cleanliness of Hospital Premises, Patient feedback on housekeeping services, Staff feedback on housekeeping services

The data could be captured using a checklist.

This need not mean routine environmental sampling.

The swabs and culture plates should not be collected routinely from OT and other high risk areas for monitoring the clean environment. Periodic fumigation is not recommended. Fogging is recommended only in following conditions

- Commissioning of new OT
- Modification/ renovation of OT
- Significant increase in SSI with evidence of involvement of environment.

Commitment g. Feedback regarding surveillance data is provided regularly to the appropriate health care provider.

Interpretation: The feedback shall include the adherence rates, healthcare associated infection (HAI) rates, trends and opportunities for improvement, including data from other surveillance activities. It could also provide specific inputs to reduce the HAI rate. This could be in the form of a bulletin/newsletter.

- Commitment h. The organisation identifies and takes appropriate action to control outbreaks of infections. ***

Interpretation: Surveillance shall help early identification of outbreaks. To define as to what constitutes an outbreak, the organisation shall have baseline rates. The organisation implements written guidance for handling such outbreaks.

- Commitment i. Surveillance data is analysed and appropriate corrective and preventive actions are taken.**

Interpretation: The Infection Prevention and Control Committee analyses the surveillance data and based on this corrective and preventive actions are taken where necessary. This also includes taking appropriate corrective actions to prevent recurrence after an outbreak.

Standard

IPC.7.

Infection prevention measures include sterilisation and / or disinfection of instruments, equipment and devices.

Objective Elements

- Commitment a. The organisation provides adequate space and appropriate zoning for sterilisation activities.**

Interpretation: Adequacy of space refers to the central sterile services department (CSSD), which shall have a suitable location, proper layout (unidirectional flow, zoning) and separation of clean and dirty areas. Sufficient space shall be available to ensure that the activities can be performed properly. It is preferable to have separate areas for receiving, washing, cleaning, packing, sterilisation, sterile storage and issue. A good reference is Hospital Infection Society India (HISI) and WHO guidelines.

CORE

- b. Cleaning, packing, disinfection and / or sterilisation, storing and the issue of items is done as per the written guidance. ***

Interpretation: The written guidance shall be in consonance with national and/or international guidelines. A good reference is "CDC Guideline for Disinfection and



CORE



Commitment



Achievement



Excellence

Sterilization in Healthcare Facilities, 2008". Other references include the Hospital Infection Society India (HISI) guidelines.

Cleaning of used instrument/equipment/device shall preferably be done in the CSSD. However, if the same is done in patient care areas, adequate measures are taken for infection prevention. Cleaning ensures that visible biological material and dirt is removed. After cleaning the instrument, sets are prepared and packed using the appropriate material. Shelf life of sterilised items is defined and monitored to increase efficacy. Spaulding's classification guides the decision to do high/intermediate/low-level disinfection. Disinfection/sterilisation is performed as per the written guidance. Flash sterilisation shall only be done in exceptional situations when there is insufficient time to sterilise an item by the preferred method. The sterilised/disinfected equipment/sets shall be stored appropriately across the organisation and not just in CSSD. The expiry date of sterilised instruments/equipment shall be guided by the packing material used and the mode of sterilisation.

Commitment c. Reprocessing of instruments, equipment and devices are done as per written guidance. *

Interpretation: The organisation identifies instruments, equipment and devices which are suitable for re-use. The number of re-uses and the process of re-use of these items are defined and monitored. The patient is informed about the same. The written guidance addresses cleaning, disinfection or sterilisation between patients. The written guidance shall be in consonance with the available good practices.

Commitment d. Regular validation tests for sterilisation are carried out and documented. *

Interpretation: This shall be done by accepted methods, for example bacteriologic, strips, etc. Physical/chemical tests shall be done daily, and biological tests at least weekly. Engineering validations like Bowie-Dick tape test and leak rate test need to be carried out.

Each load shall have a unique number and content description. Where applicable, temperature, pressure and time-record chart shall be maintained. A good reference is "CDC Guideline for Disinfection and Sterilization in Healthcare Facilities, 2008".

- Commitment e. The established recall procedure is implemented when a breakdown in the sterilisation system is identified. ***

Interpretation: The organisation shall ensure that the sterilisation procedure is regularly monitored and in the eventuality of a breakdown it has written guidance for withdrawal of such items. The organisation could have a batch-processing system with date and machine number for effective recall. The processes could be verified through a mock drill.

Standard

IPC.8.

The organisation takes action to prevent or reduce healthcare associated infections in its staff.

Objective Elements

- Commitment a. The organisation implements occupational health and safety practices as per written guidance to reduce the risk of transmitting microorganisms among health care providers. ***

Interpretation: Written guidance shall be available on occupational health and safety practices. The organisation provides adequate resources to its staff for infection prevention and control. Staff shall adhere to standard precautions. Where appropriate, the organisation shall have a pre-employment health screening and capture immunisation status.

Refer to IPC.2.c., IPC.3.a., IPC.3.b. and HRM.2.b.

- Commitment b. The organisation implements an immunisation policy for its staff. ***

Interpretation: The organisation's immunisation policy for its staff shall be in consonance with the available evidence. At a minimum, Hepatitis B vaccination shall be provided to staff involved in direct patient care. Other relevant immunisation is provided as per the risk from time to time and in accordance with applicable statutory requirements.

- Achievement c. The organisation implements work restrictions for health care providers with transmissible infections.**

Interpretation: The restrictions could include limiting role and responsibilities for health care providers with transmissible infections. The organisation encourages health care providers to report their illness or exposures and does not penalise them. Examples of transmissible infections include acute conjunctivitis, chickenpox, acute respiratory infections, smear-positive case of tuberculosis etc.

- Commitment d. The organisation implements measures for blood and body fluid exposure prevention.**

Interpretation: The organisation ensures that health care providers use appropriate personnel protective equipment to prevent blood and body fluid exposures. The organisation shall strive to eliminate the use of needle devices whenever safe and effective alternatives are available. It could consider providing needle devices with safety features.

- Commitment e. Appropriate post-exposure prophylaxis is provided to all staff members concerned.***

Interpretation: The organisation provides post-exposure prophylaxis for hepatitis B and HIV exposure. This shall align with National and/or International guidelines. Infection prevention and control Nurse maintains documentation of post-exposure prophylaxis records. For example, PEP for needle stick injury.

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